

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Patient Name: Patient 10

Age: 65 years old

Sex: Female

MRN: 000-TEST-10

Admission Date: 06/12/2026

Discharge Date: 06/18/2026

Length of Stay: 6 days

****PRIMARY DIAGNOSIS****

Congestive Heart Failure with reduced ejection fraction (HFrEF), EF 32%

****SECONDARY DIAGNOSES****

- Acute decompensated heart failure
- Rheumatoid arthritis (RA), currently stable on methotrexate
- Gastroesophageal reflux disease (GERD)
- Hypertension
- Hyperlipidemia

HOSPITAL COURSE & KEY EVENTS

Patient 10 presented via ED with progressive dyspnea on exertion (DOE), orthopnea, and lower extremity edema × 5 days. Initial vital signs showed BP 158/92, HR 104, RR 22, SpO2

89% on room air. Chest X-ray confirmed pulmonary edema with bilateral pleural effusions. BNP level on admission: 847 pg/mL (significantly elevated). Transthoracic echocardiogram revealed EF 32% with global hypokinesis and moderate mitral regurgitation.

Patient underwent aggressive diuresis with IV furosemide over 48 hours with good response — patient lost 7.2 lbs net fluid, respiratory status improved to SpO2 96% on room air by hospital day 3. Cardiology consulted and optimized medical management. Gentle ambulation begun hospital day 2; patient tolerated well. Daily weights monitored (admission weight: 156 lbs; discharge weight: 148.8 lbs). Repeat BNP on discharge: 312 pg/mL (improved). No acute coronary syndrome ruled out via serial troponins. Patient counseled extensively on fluid/sodium restriction, medication adherence, and daily weight monitoring protocol.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
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Lisinopril (ACEI)	20 mg	q.d.	ongoing
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Metoprolol Succinate (extended-release)	95 mg	q.d.	ongoing
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Furosemide	80 mg	BID	ongoing; reassess in 2 wks
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Spirolactone (aldosterone antagonist)	25 mg	q.d.	ongoing
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Atorvastatin	40 mg	q.d.	ongoing
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Omeprazole (for GERD)	20 mg	BID	ongoing
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Methotrexate (for RA)	15 mg	weekly (Thursdays)	ongoing; continue rheum schedule
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Aspirin	81 mg	q.d.	ongoing
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Potassium chloride supplement	20 mEq	q.d.	ongoing (monitor K+ levels)
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ACTIVITY & WEIGHT-BEARING STATUS

Patient cleared for light activity. Encourage slow ambulation as tolerated. NO strenuous exertion, heavy lifting, or high-intensity exercise. Resume activities of daily living (ADL) gradually. Avoid prolonged standing.

DIETARY RESTRICTIONS

****STRICT 2g sodium diet**** — NO added salt, limit processed foods, avoid cured meats.

****FLUID RESTRICTION: 1.5 liters per day**** (all fluids including water, juice, soups). Patient counseled on reading nutrition labels. Referral to dietitian provided.

DAILY WEIGHT MONITORING PROTOCOL

Weigh self each morning before